

[Portion Redacted]

Interviewer 08:21

All right, great. Now I wanted to ask you as well, I know there's bound to be our range of issues, but when clients come to you, what is usually the presenting issue or the reason that they seek your service?

Therapist 08:37

Um, the main issue is the communication. They can't reach each other, and both complain about bad communication and turning in circles around around around.

Interviewer 09:03

Oh, and by the way, do you specialize? Are there specific types of couples that you provide your necessity?

Therapist 09:11

Um, well, I have a lot of couples who are wanting to go in polyamory who are considering that. So they come with questions about how is it most didn't do it yet. There are 70%. And so we discovered the topic together and look what the pros and cons are.

Interviewer 09:46

And when the clients come to do a formal assessment.

Therapist 09:51

I have Yeah, a questionnaire and I use the ORS questionnaire like, how are they doing the four questions, and then it's come from Scott Miller.

Interviewer 10:07

Okay.

Therapist 10:10

And afterwards they rate the session.

Interviewer 10:16

I'm not that familiar with ORS, what would be in it?

Therapist 10:21

There are four questions in the beginning. How is somebody doing? So relational wise, personally, work and family? And overall? Are they sort of four questions? And then it's about the session, then it's mentioned... Well, I can send it to you if you want to.

Interviewer 10:47

It's okay.

Therapist 10:49

It's mentioned. Did we talk about the topics you like? How was the contact? The relationship? How was it overall? Yeah.

Interviewer 11:00

Okay. And are there other relationship scales that you use?

Therapist 11:07

Like about how is the relationship between the therapist and the couple?

Interviewer 11:12

Are they there between the couple that's right.

Therapist 11:15

No, no I don't do that

Interviewer 11:18

All right! So only the ORS? and how informative do you find the ORS? is it?

Therapist 11:25

For the relationship between them? No, it's more about the therapy. I assess it by how they're communicating. I can see it in the so my diagnostics is on the spot, like are they dealing with their issues. And, and I don't use a questionnaire for that.

Interviewer 11:52

Right? Why not? Why don't you use a questionnaire for that?

Therapist 11:58

Because I find personal assessment more rich. And then there is one reason and the second one is that people start to look at this assessment, and they are thinking, Oh, is it this? Or is it that their energy goes in? Oh, yeah. Hmm. I don't find that. True. Is this assessment, okay, or not? And I want to know exactly what it means. So for me, it's not so very practical. It's dividing the energy into a scale. And but on the other hand, I'm not against it, just not never used it.

Interviewer 12:44

Okay. And with ORS are there other bits, have there been any other challenges and using these scales?

Therapist 12:54

If they are challenges that I use the ORS.

Interviewer 13:00

when when when implementing the scales?

Therapist 13:04

No, no, no.

Interviewer 13:07

All right. And it is free, right? This scale?

Therapist 13:09

Yeah. Yeah.

Interviewer 13:13

All right. And so I think I believe they also touched on this in the email, but I wanted to ask you, because across the years, different therapists and different researchers have grafted as a construct of relationship quality quite differently from each other. I wanted to get your view about it for you. What is relationship quality?

Therapist 13:34

Okay, relationship quality in the couple.

Interviewer 13:38

That's right.

Therapist 13:40

What is that? That is a question. I have some points. That is being curious about the other and wanting to know their answer, before answering yourself making real time for each other week after week. Small things like giving each other a compliment every day, asking Can I do something for you? How are you? respecting each other sense of freedom and talking about that. One wants to do everything together. The other one says no. I have some friends. I want to go there. Yeah, that's another quality Yeah, really seeing each other and acknowledging the differences, but also profiting from it. And he or having humor, you know, they're very forcing.

Interviewer 15:37

maybe another angle towards asking that is for you like what differentiates a couple with a higher relationship quality with couples with low relationship quality?

Therapist 15:55

In high relationship quality, there is trust, trusting and safety that the other is telling the truth or need something you want or also Oh, that's a word. I don't know any English. I have to think about it. Am I look it up?

Interviewer 16:23

Please.

Therapist 16:40

Yeah. Yeah, to grant something to grant. Because we all have our particular hobbies and things. He likes to go with the bike, she likes to bake cake. And in high quality relationship. They acknowledge each other for that, and they thank each other. Oh, this is a nice cake, good that you work on your fit being fit and in your relationship quality do you see it as a threat? Yeah.

Interviewer 17:13

What would that word be in Dutch? If you don't mind me asking of ?

Therapist 17:18

Coonan.

Interviewer 17:20

All right. So would you say that the these are the most important things in a high quality relationship, that there is trust their safety? There's granting to each other? Would you say so?

Therapist 17:35

Yes. And humor is very important, they laugh together. And also very important, high quality relationship is that when one has a problem, he feels that he can go to the his or her partner. And they will be there for them. And they will not judge him or her. But try to find a solution or just listen. And they don't feel ashamed and it's very high quality relationships, even with very painful things.

Interviewer 18:17

So we talk about this, you know, like you, when you talk about high quality relationship, you're talking about trust about safety, about humor, etc. So there's this particular way of conceptualizing the relationship. That's how you conceptualize the relationship often differ from how your clients conceptualize like quality relationships as well. And does this matter for for therapy?

Therapist 18:46

Um, yeah, they, they I think they have a different view on that. Especially the individual you know, sometimes the man he thinks, Okay, when we have sex two times a week, then it's great relationship. And I am there also to give them a broader perspective on that and she may be want and now I'm stereotyping but it can also be in same sex couples. She may be want to go to movie or things you do with girlfriends and then I can say to her well, yeah, that is that is something you do with your girlfriends in the day think okay, that is that is a good conception of relationship. I think I also am a little bit an educator because men and women or people in general sometimes want different things and and we can see sometimes they are troubled by the day, for example, a woman she thinks, Oh, my husband, I cannot talk with him. I want to do a game Have a board game and talk and, and have some my wine and then and see movie. And there may be things you can do with with the girlfriends and don't expect so much from him. And then we can see what he all does. So my conception does matter to do some education about couples and what's right.

Interviewer 20:30

We also know that the relationship is a complex system right now than it was sex identity or relationship maintenance conflict or that during therapy, how do you decide which one to focus on? Or which one in this complex system to target?

Therapist 20:48

Yeah, well, I find sex very, very important. And it's also what as the Perell says, If you fix this issue of intimacy, intimacy, then there is that I know from experience, there was more patience, more forgiveness, in communication, because the communication becomes hard and square when when this part of intimacy is gone, because then they are both hurt, and they know this intimacy is gone. And so they are going to work it out and be communication. So that is a topic I always take by the balls, or they don't want it. I will tell them that it's an important issue.

Interviewer 21:45

All right. So sex always is an important part for—

Therapist 21:49

you. Yeah, intimacy, it's like a Yeah, it's more intimacy being intimate need to evolve into force or sexual things, but being an intimacy is having also a tactile How would you say, with each other?

Interviewer 22:10

So how do you then determine when to end with the couple there?

Therapist 22:15

Well, mostly I, I asked the couple are you? Can you go? Is this alright to go ahead? And and mostly they have some issues left? And I say, can you? Do you want to come again, or we want to fix this thing? Mostly, they can decide themselves.

Interviewer 22:41

All right. And usually from start to finish, how long does it usually take for like the successful combo therapy?

Therapist 22:51

I think five.

Interviewer 22:54

Five sessions. And usually, you give the sessions every how many weeks? So every one week, week—

Therapist 23:02

Every two weeks.

Interviewer 23:03

Every two weeks.. And how long does that usually take one hour ..?

Therapist 23:09

One hour and 15 minutes

Interviewer 23:11

Interesting. All right. So usually five sessions every two weeks. All right. So as I mentioned in my email, as well, we are currently developing a relationship quality scale that hopefully better represents a relationship as a complex system that would help therapists both in treatment planning and in treatment evaluation. How interested would you be in a state like this?

Therapist 23:39

Well, now I'm interested because we talk about it and explain it and and yeah, I would be interested in it.

Interviewer 23:51

Alright, so we want to make sure that this scale would be useful, of course to clinicians cure personally, what you'll be looking for in a newly developed scale like this, like how can we make sure that this new relationship quality scale would be useful in your practice?

Therapist 24:12

Well, what is very useful is when you have you have a scale and then Then below you say, okay, when use your score is between zero and six, you probably work on this first and then work on that. A little bit practical, I like that when they have practical implications. And the language has to be very, not very clinical. Just good for the couple also.

Interviewer 24:56

Anything else?

Therapist 25:00

It shouldn't take more than 10 minutes.

[Portion Redacted]

Interviewer 27:16

I wanted to upgrade, you're already you know, contributing to the development scale. But through this interview, but I wanted to ask if you were interested on further collaborating for the development of the scale. Practically this might mean for example, that when we finish the scale, that we we will ask you to look over it and give us your thoughts. And probably at the end of the two years as well, we wanted to scale has been developed that we will give it to you, for example, for free. And then after six months, we will ask you to report how it has been for you, whether it has been useful, whether it's at insights, etc. Would you be interested in that?

Therapist 28:04

Well, I have to think about it. I'm doing research myself, and and it's taking a lot of time for me. So I can only think about an exchange.

Interviewer 28:24

No problem at all.

Therapist 28:27

Like yeah, yeah, you know, currently, I need some people to look at my research. So ah, and I have lots of time in that. Setting a lot of time and it's so I don't think I can do it when I Yeah, it's it's it's a time issue.

Interviewer 28:52

No problem at all. I thought no worries at all. All right, talking about a time issue we are actually are wrapping up. So before we wrap up, I just like to go over the things that we have quickly talked about. So at the start, I wanted to get a better sense of your clinical practice. And you said that the treatment or the therapy modalities that you give with EFT according to variable therapy, provocative therapy and German intervention. Would you say that you equally rely on these four? Or do you rely on one more than the other?

Therapist 29:29

No, it's a mix. Maybe cognitive behavioral therapy the least.

Interviewer 29:35

All right. And then [portion redacted]. And you have a quite a bit of clients as well, trying to explore polyamory. When clients come to you the usual presenting issue with communication that they can reach out to each other and then they complain about this negative, just like cycles of interactions. You said that You do a formal assessment through the ORS with this kind of scale. And you're really it's more about for the therapy, or what they're trying to get out of therapy rather than their relationship with each other. Because your your diagnostic or use on the spot, because you believe that, you know, your personal assessment is more rich. And then it also when you use a scale, the energy tends to be divided. So for you, you don't want that, but you're not against it. It's just that you know, how you have been doing work for you. Oh, let me let me talk about relationship quality at for you, you brought up some points freed up to be curious, wanting to know their answer before answering itself, making time for each other week after week, giving compliments, asking them if they can do something for each other, how they are respecting each other's sense of freedom, talking about talking to each other, seeing their humor, seeing differences and appreciating the differences. And you also said about trust. And for example, what differentiates couples with higher relationship quality and relationship quality is trust, safety, granting to each other. And it's also very important to again, the humor laughing together, and that they can go to their partner, for example, without judgment, without shame, even for painful, painful things. Would these things be similar as well to your clients who want to go into polyamory or polyamorous relationships? Are these the same things that matter for them as well?

Therapist 31:47

Then communication is so much more important. Knowing how to communicate and to express your own feelings, knowing them, expressing them, and then working them through that is very hard.

Interviewer 32:10

There are other would there be any other differences in how you would approach for example, polyamorous, polyamorous clients with traditional couples?

Therapist 32:25

No, it's this communication issue is very, very important, also expressing the feelings and if one of the partners can't do it, then I will say, Well, this will be one difficult thing, you know, there is no security. So the only security is always yourself, if you feel like the other relationship is a threat, you have to be able to express that. And to tell the other why. And this is quite an advanced communication skills.

Interviewer 33:05

All right. And then I also asked about the how you conceptualize relationship and whether it is different from how your clients can sense the relationship. And for you, what you said is that you also serve in the role of an educator, for example, whether activities are better left for their friends, or further boyfriends. So for you, it does impact the sort of therapy, we also talked about how you target which in the relationship to which in the complex systems, the darker and for you, really, one of the things that are really important is intimacy. Because if there's intimacy, there's more patience, more forgiveness, more communication, etc. And then usually, the treatment from start to finish takes about five sessions every two weeks, one hour and 15 minutes. Or you are interested in the scale like this, it will be useful for you, for example, if it has practical implications, if a certain score, for example, will say that, okay, we're gonna do this, or prioritize this. You want the language to not be very clinical, and it shouldn't take 10 minutes, more than 10 minutes. And I believe that's it. Is there anything you'd like to add? Anything you'd like to add to that?

Therapist 34:25

No..Okay. And if you want, you can contact me in half a year. Maybe things are different with my time.

Interviewer 34:38

All right. Definitely. This would be....

Therapist 34:43

what is important. It's all anonymous. Yeah

Interviewer 34:47

That's I wanted to talk to you about that actually, about how we will handle the data. I wanted to know which data sharing option you are more comfortable with, of course, as a research project, we want to be as transparent As ethically possible, so one option for this is that because I will turn this interview into a transcript, one option can be transcript portions can be shared, none of which can be identified to you. Another option, if you were bolder, is that the whole transcript can be shared, removed or sensitive and overly identifying details. So it will be anonymized. Although in this option, there is a minimal chance that people who know you professionally well can piece together the details in the transcript that will Oh, provocative therapy in the [Country] that they might be together that that few so of the two data sharing options, which one are you more comfortable with?

Therapist 35:48

The first one.

Interviewer 35:50

The first one all right. All right, so I have to end our interview, actually Thank you so much.

Therapist 35:59

Thank you [Name].

Interviewer 36:02

Sorry, by the way before, and because we are still looking for more participants, particularly from you know, especially with experience in polyamory, LGBTQ and might potentially not in Europe as well so either your us or your or not European with you know, contacts with you know, other relationship therapist who might be interested?